

Building the Watchdog Before the House Burns Down:

Forensic Governance Architecture for NHI Implementation

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Between April 2020 and August 2021, South Africa lost an estimated R14.3 billion to irregular, wasteful, and fruitless expenditure in its COVID-19 procurement response.^{1,2} The Special Investigating Unit's findings, tabled before Parliament, did not reveal a system attacked from outside. They revealed a system designed without the internal architecture to prevent predation from within. The PPE crisis was not primarily a story of criminals. It was a story of governance infrastructure that arrived after the money — and therefore arrived too late.

The National Health Insurance Fund, once fully operational, will manage a budget conservatively projected to exceed R200 billion annually. It will serve as the single purchaser and single payer of health care services for every South African citizen, permanent resident, and qualifying refugee.³ It will maintain the largest health database on the African continent. It will accredit thousands of public and private providers, process millions of claims, and determine benefits for a population of more than 60 million people. The question this article does not ask is whether the NHI is necessary or constitutionally grounded — it is both. The question this article asks is simpler and more urgent: when the Fund becomes operational, will the watchdog already be inside the house?

The Constitutional Promise and the Governance Obligation

The National Health Insurance Act 20 of 2023 is anchored in an unambiguous constitutional commitment.³ Section 27 of the Constitution guarantees every person the right of access to health care services.⁴ The Act's Preamble explicitly recognises the socio-economic injustices and inequities of the past, and frames the NHI as an instrument of social solidarity — the deliberate pooling of risk across the young and old, the wealthy and the poor, the healthy and the sick. This is not administrative policy. It is a constitutional obligation in legislative form.

The very weight of that obligation generates a forensic governance imperative that the Act's architects understood in principle but did not fully resolve in text. The populations most dependent on NHI succeeding are precisely those with no private sector alternative — communities that have historically borne the full cost of public sector governance failure. When a medical scheme fails, its members seek alternatives. When the NHI Fund fails, its most vulnerable users cannot. The constitutional promise of universal access is therefore inseparable from the obligation to build governance architecture capable of protecting it. To treat forensic governance as a secondary implementation concern is not a technical oversight. It is a failure of constitutional stewardship.

Three Structural Vulnerabilities in the Act's Text

The NHI Act creates a governance architecture of genuine ambition. It establishes the Fund as an autonomous public entity under the Public Finance Management Act,^{3,10} subjects it to annual Auditor-General oversight under section 50, and requires quarterly reporting to the Board and annual reporting to Parliament under section 20. These are not weak accountability mechanisms. But accountability mechanisms that operate after the fact — audit, reporting, parliamentary scrutiny — are necessary conditions for governance, not sufficient ones. Between the moment money enters the Fund and the moment an audit flags its misuse, there is a space. In large, complex health systems, that space is where systemic harm lives. Three

provisions in the Act create structural conditions that widen that space in ways that deserve urgent professional attention.

Gap One — The Accreditation Bottleneck (Section 39)

Section 39 establishes accreditation as the sole gateway to payment.³ No provider may be reimbursed by the Fund unless accredited. To be accredited, a health care service provider or health establishment must, among other requirements, produce proof of certification by the Office of Health Standards Compliance — the body established under section 77 of the National Health Act to inspect and certify health establishments against prescribed norms and standards.¹¹ Its capacity constraints are not speculative — they are documented across successive Auditor-General reports and OHSC annual reporting.^{5,6} The NHI Act now places that same institution at the bottleneck of a national accreditation system covering every public and private provider seeking Fund reimbursement, simultaneously, at scale.

Governance theory and South African procurement history converge on a predictable outcome when accreditation bottlenecks meet financial pressure: ghost accreditation, where certification reflects administrative processing rather than genuine compliance assessment, and fronting, where accredited entities serve as conduits for unaccredited providers. Section 39 contains no provision for phased OHSC capacity building, no interim oversight mechanism for the transition period, and no forensic governance protocol for detecting either failure mode during Phase 1. The bottleneck is not a policy failure. It is written into the text.

Gap Two — The Information Platform's One-Sentence Fraud Architecture (Section 40)

Section 40 establishes the Fund's information platform — a system designed to support population health needs assessment, financing, purchasing, patient registration, provider contracting, reimbursement, utilisation monitoring, and performance management.³ In practical terms, this platform will become the largest health database on the African continent. It will

hold biometric registration data for every user, clinical records, diagnosis and procedure codes, medicine dispensing records, inpatient length of stay data, referral pathways, and provider payment histories.

The scale of this data architecture is not the concern. The concern is section 40(5), which addresses fraud and risk management in its entirety as follows: “The information architecture must include a fraud and risk management mechanism.” Eleven words. For a system that will process payment determinations, benefits eligibility decisions, and provider reimbursements for more than 60 million people, the Act provides one sentence. There is no methodology requirement. There is no specification of independence from executive management. There is no external audit protocol. There is no requirement for algorithmic accountability mechanisms where automated decision-making is used in claims processing or benefits determination.

As claims volumes scale toward full implementation, automated processing will be operationally necessary. Algorithmic decision-making in benefit determinations creates governance risks that conventional financial audit is not designed to detect — systematic bias, coding pattern manipulation, and structured exploitation of reimbursement formulae that fall below individual audit materiality thresholds but cumulatively represent significant fund leakage. The EU AI Act classifies AI systems used in health care benefit determination as high-risk, requiring mandatory conformity assessments, transparency obligations, and human oversight before deployment.⁸ South African law contains no equivalent framework.⁹ Section 40(5) does not acknowledge the gap. One sentence is not a governance framework. It is a placeholder.

Gap Three — The Undefined Investigation Unit (Section 20(2)(g))

Section 20(2)(g) requires the Chief Executive Officer to establish a Risk Management and Fraud Prevention Investigation Unit within the Fund's national office.³ The unit's mandate is substantive and necessary — investigating complaints of fraud, corruption, criminal activity,

unethical business practices, and abuse relating to any matter affecting the Fund or its users. The problem is not what the unit is required to do. The problem is that the Act is entirely silent on how it must be constituted to do it.

Section 20(2)(g) specifies no professional competency standards. It provides no independence safeguard separating the unit from the executive management it may be required to investigate. It contains no requirement for external forensic governance expertise or defined escalation protocols to the SIU, the Hawks, or the National Prosecuting Authority.

The Zondo Commission's evidence base documents this failure pattern extensively.⁷ Internal investigation units in major public entities — established with genuine legislative mandate — were systematically hollowed out through under-qualified appointments, resource denial, and executive interference. The pattern was a predictable consequence of establishing the mandate without establishing the architecture. Section 20(2)(g) repeats that structural choice. A unit with a broad mandate, no professional standards, and no independence safeguards is not an oversight instrument. It is an oversight symbol.

What Forensic Governance Architecture Means in Practice

The three gaps identified above share a common structural feature. They are not gaps in the Act's ambition — they are gaps in the pre-implementation design of detection, accountability, and escalation mechanisms. This distinction determines the nature of the solution. They are implementation architecture deficiencies — and implementation architecture can be designed, built, and embedded during Phase 1 without amending a single provision of the Act.

Forensic governance architecture, as a professional discipline, is distinct from financial auditing, compliance monitoring, and clinical governance. Financial auditing operates retrospectively on recorded transactions. Compliance monitoring measures adherence to defined rules. Clinical governance focuses on care quality outcomes. Forensic governance

architecture operates upstream of all three — the deliberate design of systems that make misconduct detectable before it becomes entrenched, accountability structures that function under institutional pressure, and escalation pathways that connect ground-level anomaly detection to consequential oversight. Three concrete architectural interventions are proposed, each directly responsive to a gap identified above and each implementable within the Phase 1 timeline that section 57 defines.

Proposal One — An NHI Forensic Governance Protocol (Responsive to Section 20(2)(g))

The Risk Management and Fraud Prevention Investigation Unit should be constituted against a formally documented Forensic Governance Protocol developed before the unit is staffed and operational. This Protocol should define the professional competency standards required for unit membership — drawing on existing frameworks from the Association of Certified Fraud Examiners, the South African Institute of Chartered Accountants' forensic practice standards, and the Health Professions Council of South Africa's clinical governance requirements as an intersection rather than a selection of one. It should specify the unit's methodology for complaint intake, evidence preservation, investigation conduct, and referral to external bodies. It should establish independence safeguards including a direct reporting line to the Board's audit and risk committee that bypasses executive management for investigations involving Fund leadership. A Ministerial directive under section 56 already empowers this. The mechanism is in the text. What is absent is the political and professional will to use it before the unit's first failure rather than after.

Proposal Two — Forensic Governance Impact Assessments at Implementation Milestones (Responsive to Section 39)

Before each phase of accreditation rollout is activated, a Forensic Governance Impact Assessment should be conducted and its findings tabled before the Board. The concept is analogous to the environmental impact assessment framework in infrastructure development

— a mandatory pre-implementation analysis that identifies risk surfaces, quantifies exposure, and specifies mitigation measures before financial commitments are made. Applied to NHI accreditation, each assessment would analyse the specific fraud vectors associated with the provider category being accredited, assess the OHSC's certification capacity relative to the volume of providers entering that phase, identify the bottleneck points where accreditation pressure is most likely to generate ghost accreditation and fronting risks, and specify the monitoring protocols the Fund's information platform must have operational before the phase commences. The PFMA's risk management requirements under section 38 already create the legislative foundation for mandatory risk assessment before significant financial commitments.¹⁰ The Forensic Governance Impact Assessment is an application of that existing framework to the specific risk architecture of NHI accreditation.

Proposal Three — An AI Audit Protocol for the Information Platform (Responsive to Section 40)

Before the information platform is deployed at operational scale, an AI Audit Protocol should be embedded in its design architecture. Drawing on the EU AI Act's risk classification framework,⁸ this Protocol should establish at minimum: a mandatory independent conformity assessment before deployment; transparency obligations ensuring any automated determination affecting benefit entitlement is capable of human-readable explanation — the right to written reasons in section 6(j) of the Act is meaningless if the determination was generated by an algorithm that cannot explain itself; human oversight mechanisms for determinations above defined financial thresholds or involving vulnerable population categories; and annual algorithmic audits conducted by a party independent of both the Fund and the platform's technology provider. South Africa's POPIA framework provides the data protection foundation.⁹ An AI Audit Protocol is the operationalisation of that framework in the specific context of automated health benefit determination. It is not regulatory overreach. It is POPIA compliance made concrete.

The Professional Ecosystem Gap

The three proposals above each require a professional competency that does not currently exist as a formally recognised practice category in South Africa. Financial auditors, compliance officers, clinical governance specialists, and forensic investigators each occupy defined professional spaces with established credentialing frameworks and regulatory recognition. The NHI system requires their intersection — a practitioner capable of reading a clinical claims pattern as both a governance anomaly and a potential fraud indicator, of understanding a provider accreditation decision as both a regulatory act and a financial risk event, of designing an algorithmic audit protocol that is simultaneously technically sound, legally grounded, and clinically literate.

That intersection is not currently occupied as a formal professional discipline in South Africa. This is not a minor gap in a mature system. It is a foundational capacity deficit in a system that has not yet been built — which means the window to establish the professional framework before the system demands it remains open. The health policy community, professional councils, and academic institutions that shape South Africa's governance ecosystem carry a shared responsibility to name and close that deficit during Phase 1, before Phase 2's operational demands make reactive improvisation the only available option.

Conclusion

Section 57 of the National Health Insurance Act places the current moment inside Phase 1 — the four-year period from 2023 to 2026 in which governance structures are to be established, institutions founded, and the operational architecture of the Fund designed.³ That window is not unlimited. Phase 2 begins the selective contracting of health care services and the full operationalisation of the Fund as a mandatory prepayment purchaser. Once that transition begins, the governance architecture that exists will be the governance architecture the system operates with.

The NHI represents the most significant restructuring of a public system in South Africa since the democratic transition of 1994. Its constitutional grounding is sound. Its social solidarity logic is compelling. Its potential to begin correcting thirty years of health inequality is real. None of that potential is served by a governance architecture that arrives after the first crisis rather than before it. The house is being built. The watchdog must be inside it before the doors open — not summoned after the first fire.

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Author Declarations

Author Contributions

WW Matthee conceived the article, conducted the statutory analysis, developed the forensic governance architecture framework, drafted the manuscript in full, and approved the final version for submission.

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Conflict of Interest Declaration

The author is the founder of EthicHawks, a forensic governance and compliance consultancy positioned to provide services in the healthcare governance sector, including services potentially relevant to NHI implementation. This commercial interest is declared in full. The proposals advanced in this article are grounded in the Act's own text and publicly available evidentiary record. The author has no current contractual relationship with the NHI Fund, the Department of Health, or any party directly involved in NHI implementation at the time of submission.

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